PAMPATTIWAR, VIVEK DOB: 11/10/1969 (55 yo M) Acc No. 13717 DOS: 01/09/2025

Consultation Notes for VIVEK PAMPATTIWAR on 01/09/2025

History of Present Illness

Patient is a 55 yo Indian American who presented in our clinic today for the following problems:

1. back pain_ axial pain

Location of pain: lower back. Right lower leg cramps randomly. worse when he used R leg for driving. denied family history of neuropathy, myopathy. routine electrolyte tests were normal. He is a vegetarian.

Onset: chronic for 4-5 years and progressive worsen in last 3 months

VAS is: 6-8/10

Duration: varies.

Quality of the pain: The patient describes the pain as dull, spastic, burning and aching

W/: Worse by twisting, extension, standing and walking.

B/: Some pain relief by taking oral and topical analgesics, and bed rest.

PT/home exercise Not effective

MRI of lumbar spine was not performed

2. leg pain and heaviness with worse pain after walking for a short distance. Patient had to sit down for rest before walking again.

Patient reports he already went to vein clinic few years ago and ultrasound was negative.

Patient denied bowel or bladder dysfunction.

Patient has history of hypertension, hyperlipidemia, and diabetes.

Patient denied history of smoking, heavy drinking or illicit drug use.

Vital Signs

Pain scale: 10 1-10, Temp: 98.7 F, BP: 133/83 mm Hg, HR: 73 /min, Wt: 118 lbs, Ht: 62 in, BMI: 21.58 Index, Wt-kg: 53.52 kg.

Examination

General: Well nourished, well developed, cooperative. Cardiac: Regular rate and rhythm.

Neuro: awake, alert, oriented to person, time and place. Language fluent. CN II-XII grossly intact. Motor and sensory exam are grossly normal except mild decreased sensation in leg.

Spine: Tenderness at lumbar local region as patient described in HPI. Scooped posture and wide based gait with slight limping.

Other positive special tests include:

- Lumbar facet pain causing decreased ROM, Lumbar facet loading test
- Lumbar radiculopathy / sciatica pain causing Straight leg raise (SLR) test, Sciatic notch tenderness.

Treatment Plan & Orders

1. Spondylosis without myelopathy or radiculopathy, lumbar region (M47.816)
   Imaging: MRI : Lumbar without contrast
2. Spinal stenosis, lumbar region with neurogenic claudication (M48.062)
   Imaging: MRI : Lumbar without contrast
3. Idiopathic peripheral neuropathy (G60.9) - EMG 95886 TWO units inHouse

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